

**HOW TO USE THIS SET**

20 questions, mixed difficulty, 9 case-based. Attempt every question before checking the answer key on the last page.

4 Easy

10 Medium

6 Hard

1.**EASY**

Which organism is the named cause of secondary achalasia?

- A. *Helicobacter pylori*
- B. *Trypanosoma cruzi*
- C. *Candida albicans*
- D. Herpes simplex virus
- E. *Schistosoma*

2.**EASY**

Esophageal varices are caused by cirrhosis and portal hypertension in what proportion of cases?

- A. Under 25%
- B. About 50%
- C. About 70%
- D. Over 90%
- E. 100%, with no other cause

3.**EASY**

Which esophageal carcinoma is more common worldwide overall?

- A. Adenocarcinoma
- B. Squamous cell carcinoma
- C. Leiomyoma
- D. Papilloma
- E. Both are equally common worldwide

4.**EASY**

What is the commonest presenting symptom of esophageal carcinoma?

- A. Odynophagia
- B. Haematemesis
- C. Dysphagia
- D. Weight loss alone
- E. Chest pain mimicking cardiac disease

5.**MEDIUM****CASE**

A 34-year-old man from a rural Brazilian region has progressive dysphagia. Manometry shows raised lower esophageal sphincter tone, incomplete LES relaxation, and aperistalsis of the esophageal body.

Which organism should specifically be considered in this patient's history?

- A. *Candida albicans*
- B. Herpes simplex virus
- C. *Trypanosoma cruzi*
- D. Human papillomavirus
- E. *Helicobacter pylori*

6.

MEDIUM

CASE

Two patients vomit repeatedly after a heavy drinking episode. One develops haematemesis alone; the other develops sudden severe chest pain, tachypnoea, and shock.

What structurally distinguishes the second patient's lesion from the first's?

- A. It occurs in the proximal esophagus rather than distally
- B. It is a transmural tear rather than a mucosal one
- C. It results from portal hypertension rather than retching
- D. It is caused by a different underlying mechanism entirely
- E. It occurs only in patients with pre-existing varices

7.

MEDIUM

CASE

An immunosuppressed patient develops odynophagia. Endoscopy shows adherent grey-white plaques over the esophageal mucosa.

What is the most likely cause?

- A. Candida albicans
- B. Herpes simplex virus
- C. Eosinophilic esophagitis
- D. Reflux esophagitis
- E. Graft-versus-host disease

8.

MEDIUM

CASE

A biopsy from the same immunosuppressed patient's neighbouring lesion instead shows shallow, punched-out ulcers with multinucleate squamous cells containing nuclear inclusions.

This second lesion is best explained by:

- A. Candida albicans
- B. Herpes simplex virus
- C. Eosinophilic esophagitis
- D. Bullous pemphigoid
- E. Chemical esophagitis

9.

MEDIUM

CASE

An atopic child has dysphagia. Endoscopy shows transverse ridges resembling the trachea, and biopsy shows numerous intraepithelial eosinophils concentrated far from the gastro-oesophageal junction.

Which single feature most argues against a diagnosis of reflux esophagitis here?

- A. The presence of eosinophils at all
- B. The patient's young age
- C. The location of the eosinophils far from the GEJ
- D. The presence of basal zone hyperplasia
- E. The presence of transverse ridges

10.

MEDIUM

A 46-year-old man with long-standing heartburn has a biopsy showing acquired glandular epithelium replacing squamous mucosa above the gastro-oesophageal junction. He is now at increased risk of which malignancy, and where does it typically arise?

- A. Squamous cell carcinoma of the middle third
- B. Adenocarcinoma of the distal third
- C. Leiomyoma of the lower third
- D. Squamous cell carcinoma of the upper third
- E. Adenocarcinoma of the middle third

11.

MEDIUM

Which of the following lowers LES tone or raises intra-abdominal pressure, thereby predisposing to GERD?

- A. Achalasia
- B. Hiatal hernia
- C. Herpes esophagitis
- D. Candidiasis
- E. Mallory-Weiss tear

12.

MEDIUM

CASE

A patient with a tumour in the middle third of the esophagus is being staged.

Which nodal group is most likely involved first?

- A. Cervical nodes
- B. Mediastinal, paratracheal, and tracheobronchial nodes
- C. Gastric and coeliac nodes
- D. Inguinal nodes
- E. Axillary nodes

13.

MEDIUM

The commonest benign tumour of the esophagus is:

- A. Papilloma
- B. Leiomyoma
- C. Pleomorphic adenoma
- D. Adenoma
- E. Lipoma

14.

MEDIUM

Which risk factor is specific to squamous cell carcinoma of the esophagus rather than adenocarcinoma?

- A. Barrett esophagus
- B. Chronic GERD
- C. Very hot drinks and caustic injury
- D. Obesity
- E. Hiatal hernia

15.

HARD

CASE

A patient has known Barrett esophagus. Biopsy this year shows glandular mucosa with architectural crowding and cytological atypia confined to the epithelium, without invasion.

Where does this finding sit in the progression toward adenocarcinoma?

- A. It is the earliest stage — Barrett without dysplasia
- B. It represents dysplasia, the intermediate stage between metaplasia and invasive carcinoma
- C. It is already invasive adenocarcinoma
- D. It indicates regression back to normal squamous mucosa is still possible spontaneously without any monitoring
- E. It is unrelated to the Barrett sequence and should be treated as reflux esophagitis alone

16.

HARD

Which statement correctly distinguishes eosinophilic esophagitis from reflux esophagitis when both show intraepithelial eosinophils?

- A. Only reflux esophagitis ever shows eosinophils; their presence excludes eosinophilic esophagitis
- B. Eosinophil number and distance from the GEJ separate them — few and near the GEJ favours reflux, numerous and distant favours eosinophilic esophagitis
- C. Basal zone hyperplasia occurs only in eosinophilic esophagitis
- D. Transverse ridging is diagnostic of reflux esophagitis, not eosinophilic esophagitis
- E. Atopy excludes a diagnosis of reflux esophagitis

17.

HARD CASE

A patient has esophageal varices measuring 3 mm on endoscopic survey, with intact overlying mucosa.

Based on the size threshold associated with rupture risk, which statement is most accurate?

- A. These varices are already above the size threshold linked with rupture
- B. These varices are below the size threshold most linked with rupture, though risk is never zero
- C. Varix size has no bearing on rupture risk, only portal pressure does
- D. Intact mucosa means these varices cannot be a source of bleeding under any circumstance
- E. Varices under 5 mm never require endoscopic surveillance again

18.

HARD

Which single feature is shared by the progression GERD→Barrett→adenocarcinoma, H. pylori→intestinal metaplasia→gastric cancer, and adenoma→colorectal cancer?

- A. All three require an infectious trigger
- B. All three follow a chronic injury → metaplasia/dysplasia → carcinoma sequence
- C. All three are driven primarily by an inherited mutation
- D. All three arise exclusively in squamous epithelium
- E. All three are unrelated processes that happen to occur in the GIT

19.

HARD CASE

A patient has odynophagia and endoscopy shows shallow punched-out ulcers. The clinician is deciding between herpes esophagitis and candidal esophagitis before biopsy results return.

Which finding, if present on gross endoscopy, would most favour candidal over herpetic esophagitis?

- A. Multinucleate squamous cells on scraping
- B. Adherent grey-white pseudomembranes rather than discrete ulcers
- C. Nuclear inclusions on cytology
- D. Vesicles preceding the ulcers
- E. A history of recent organ transplantation, since this only predisposes to herpes

20.

HARD CASE

A patient with known Barrett esophagus has surveillance biopsies showing high-grade dysplasia confirmed by two pathologists, with no evidence of invasion on imaging.

Where does this sit relative to invasive adenocarcinoma in the progression sequence, and what does that imply?

- A. It is equivalent to invasive adenocarcinoma and should be treated identically
- B. It is the step immediately preceding invasive adenocarcinoma — the highest-risk point before invasion occurs
- C. It is a reversible, low-risk finding requiring no closer surveillance than low-grade dysplasia
- D. It indicates the process has regressed toward Barrett without dysplasia
- E. It is unrelated to cancer risk and reflects reflux activity only

ANSWER KEY & RATIONALE

Don't peek until you've committed to an answer on every question.

Q	Ans	Why
1	B	Trypanosoma cruzi (Chagas disease) is the named secondary cause of achalasia — it destroys the myenteric plexus.
2	D	Cirrhosis and portal hypertension account for esophageal varices in over 90% of cases.
3	B	SCC is more common worldwide overall, though adenocarcinoma predominates in the US and Western countries.
4	C	Dysphagia is the commonest presenting symptom of esophageal carcinoma, though it typically appears late.
5	C	Chagas disease (T. cruzi) produces secondary achalasia with the same triad as the primary, idiopathic form.
6	B	Boerhaave syndrome is a transmural tear (vs the mucosal-only Mallory-Weiss tear); same triggering mechanism, very different depth and outcome.
7	A	Adherent grey-white pseudomembranes of matted hyphae are the identifying finding of candidal esophagitis.
8	B	Punched-out ulcers with multinucleate cells and nuclear inclusions are classic for herpes (HSV) esophagitis.
9	C	Eosinophils occur in both conditions; their number and distance from the GEJ (numerous and far from the GEJ) is what favours eosinophilic esophagitis over reflux.
10	B	This is Barrett esophagus; the resulting adenocarcinoma characteristically arises in the distal third.
11	B	Hiatal hernia lowers LES competence/raises pressure gradients and predisposes to GERD; the others are not GERD risk factors.
12	B	Middle-third tumours drain to mediastinal, paratracheal, and tracheobronchial nodes, following the local lymphatic anatomy.
13	B	Leiomyoma is the commonest benign esophageal tumour (mesenchymal); papilloma is the benign epithelial tumour but less common overall.
14	C	Very hot drinks and caustic injury are SCC-specific risk factors; Barrett, GERD, obesity, and hiatal hernia all point toward adenocarcinoma.
15	B	Confined, non-invasive cytological atypia in Barrett mucosa is dysplasia — the intermediate stage between metaplasia and invasive adenocarcinoma.
16	B	Both conditions can show eosinophils; the number and distance from the GEJ is the discriminator, not presence/absence alone.
17	B	Varices are considered at meaningfully increased rupture risk above 5 mm; 3 mm sits below that threshold, though no size is entirely risk-free.
18	B	All three follow the same shape: chronic injury → metaplasia → dysplasia → carcinoma — the block's recurring pattern.
19	B	Grey-white pseudomembranes (vs discrete vesicular ulcers) point toward Candida rather than HSV on gross appearance alone.
20	B	High-grade dysplasia is the step immediately before invasive adenocarcinoma — the point of highest risk within the Barrett sequence, distinct from both low-grade dysplasia and frank invasion.

Pathology GIT Block · MEDucation by Mattin Majid · MCQs written to test LG2 content — not sourced from a past paper.

Recalled and source questions are treated as evidence of what gets asked, not as a source of correct answers — verify against your lecture notes.